Personal Genomic Data Sharing:

Current Challenges & Future Possibilities

Elias Crum

VITO Digital Precision Health Day
October 10th, 2025

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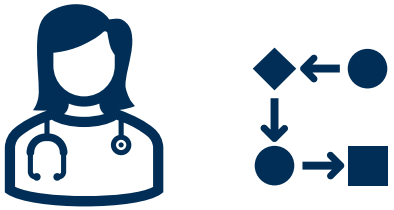
The Main Challenges (for clinical use)

Clinical Workflow Integration



The Genomic Data

The Regulatory Landscape

Technical Implementation

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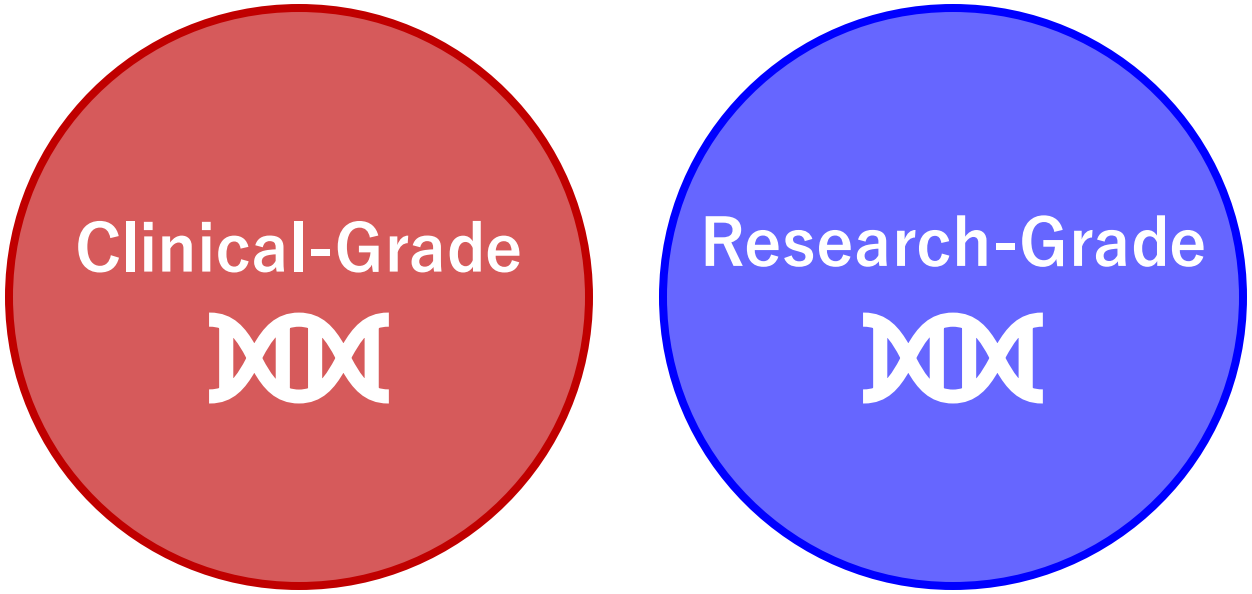
Genomic Data for Clinical Use

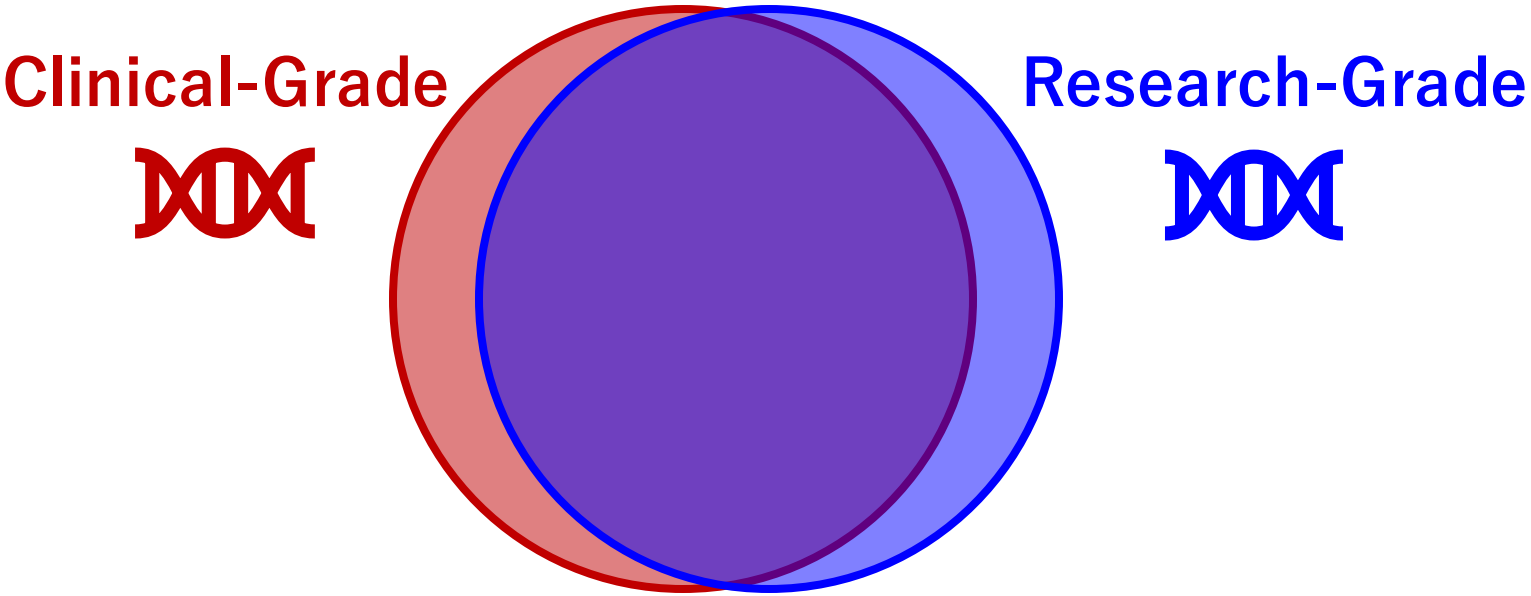
- Format is use-case specific
- Whole Genome Sequence (WGS) data
- Large Projects: 1+ Million Genomes (EU) [1]



[1] https://digital-strategy.ec.europa.eu/en/policies/1-million-genomes

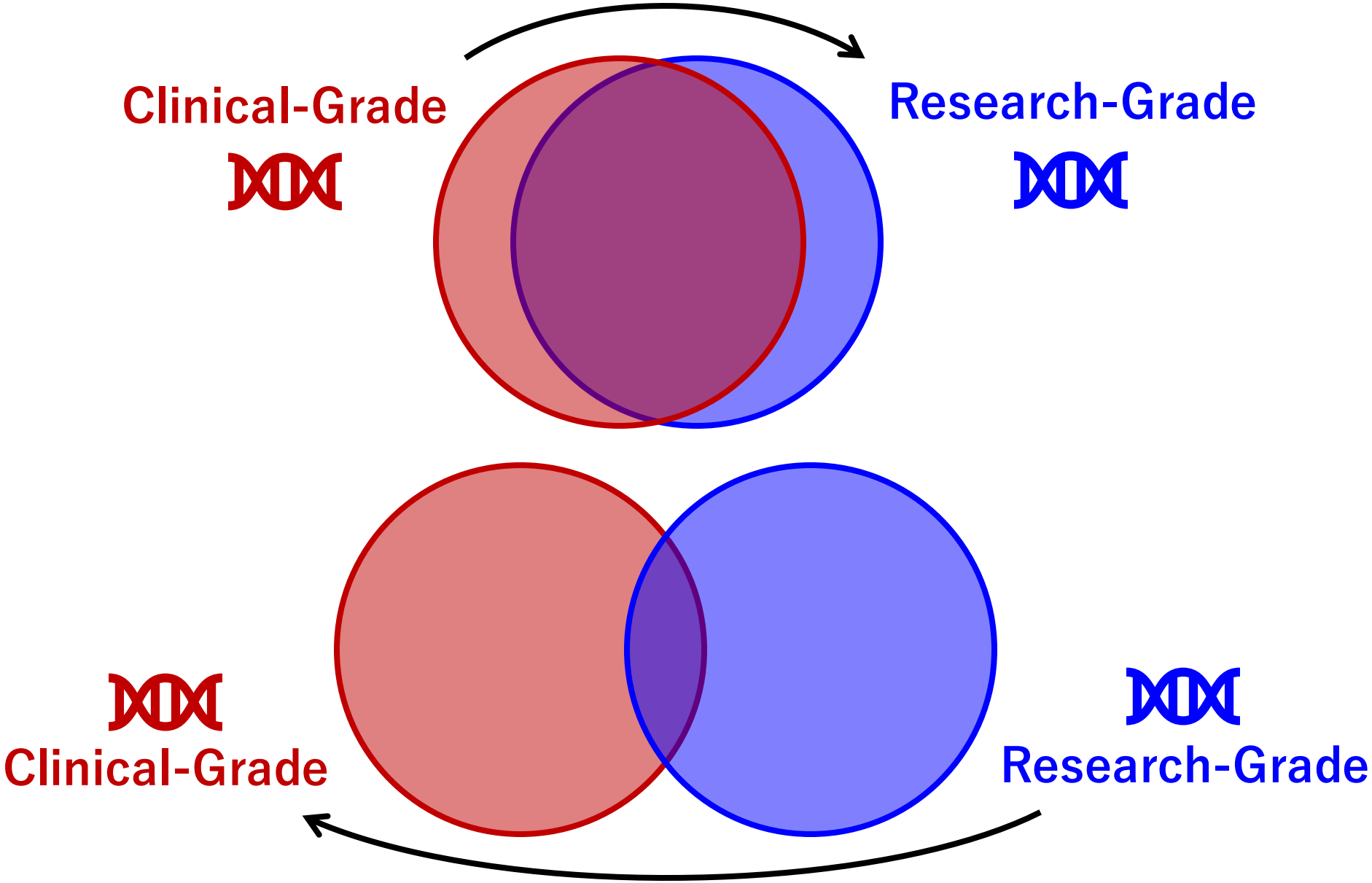
Genomic Data for Clinical Use (assumption)

- Format is use-case specific
- Whole Genome Sequence (WGS) data
- Large Projects: 1+ Million Genomes (EU) [1]



[1] https://digital-strategy.ec.europa.eu/en/policies/1-million-genomes

Genomic Data for Clinical Use (reality)



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The current landscape

Intended use is VERY important

Clinical use ≠ Research use

** Funding goes predominantly to RESEARCH-GRADE **

- Cheaper
- Fewer regulatory challenges
- Funders / Policy makers misunderstanding

** But how do we “translate” to clinical use?? **

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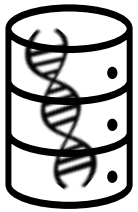
Sharing Genomic Data – Technology Considerations

Key Implementation Factors



Data Storage

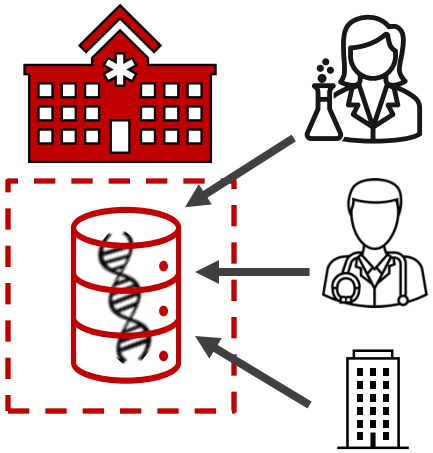
Considerations:

- patient consent
- data security
- access controls

&

Considerations:

- consent changes
- governance
- infrastructure



Data Sharing

A more in-depth analysis of these challenges + potential solutions detailed in an upcoming publication in *Computers in Biology and Medicine*

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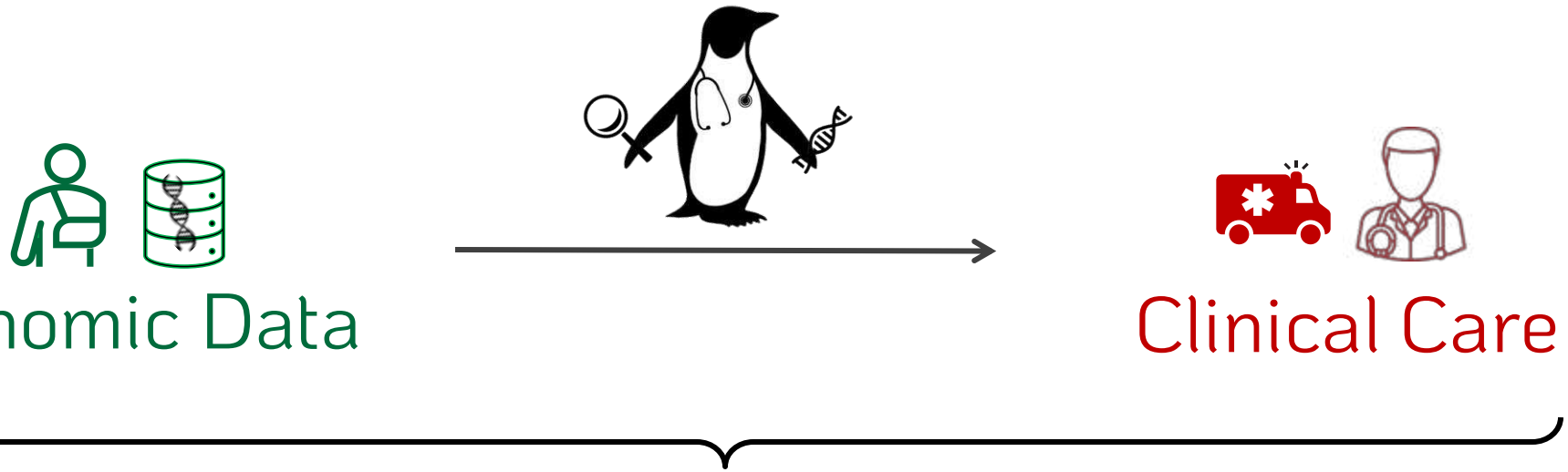
PENGQUIN: PErsoNal Genome QUery IN healthcare and clinical practice



Framework + App

Semantic Representation

Query Capabilities

1. Genomic Data Interaction

2. Semantify Genomic Data

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1. Data Interaction Framework

A Web Application to:

- *Upload Data to a Solid Pod*
- *Browse contents of a Solid Pod*
- *Edit Privacy of (and share) data in a Solid Pod*
- *Query Data in Solid Pods + SPARQL Endpoints*
  - *Includes a query cache for storing queries and results*

*** Enables patient, provider, and auditor control over data ***

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What is a Solid Pod?

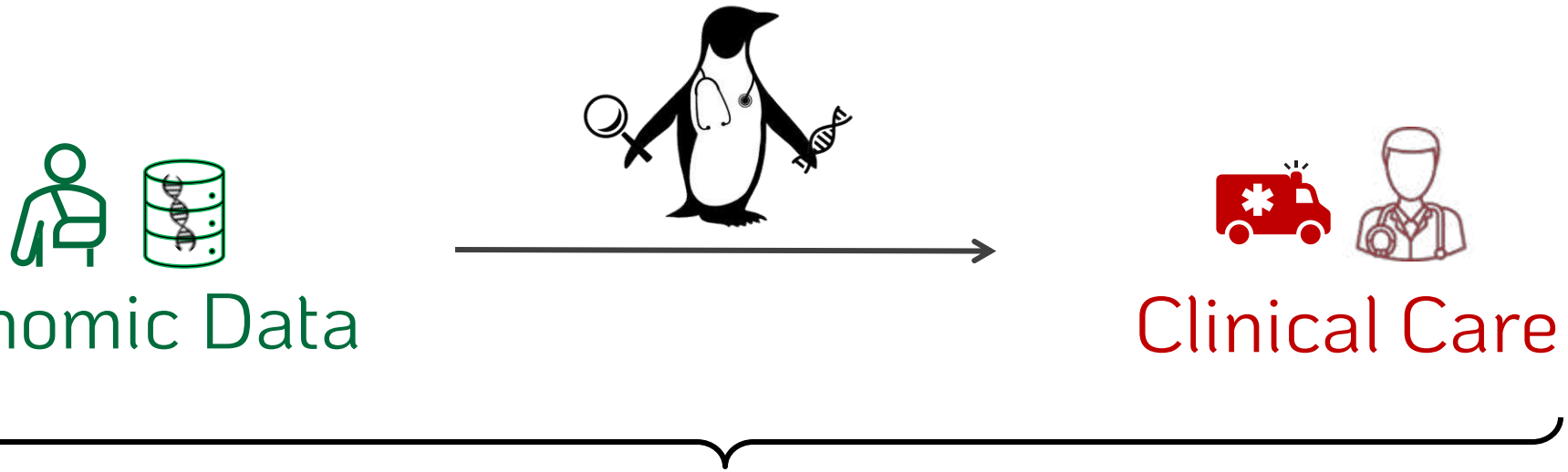
Secure, Decentralized Data Storage Specification

- *Pod exists on a Server (accessible to the Web)*
- *Can store files, RDF data, and Linked Data*
- *Provides control policies for stored data*

*** Enables secure, authenticated sharing via HTTP protocols ***

https://solidproject.org/about

PENGQUIN: PErsoNal Genome QUery IN healthcare and clinical practice



Framework + App

1. Genomic Data Interaction

Semantic Representation

2. Semantify Genomic Data

Query Capabilities

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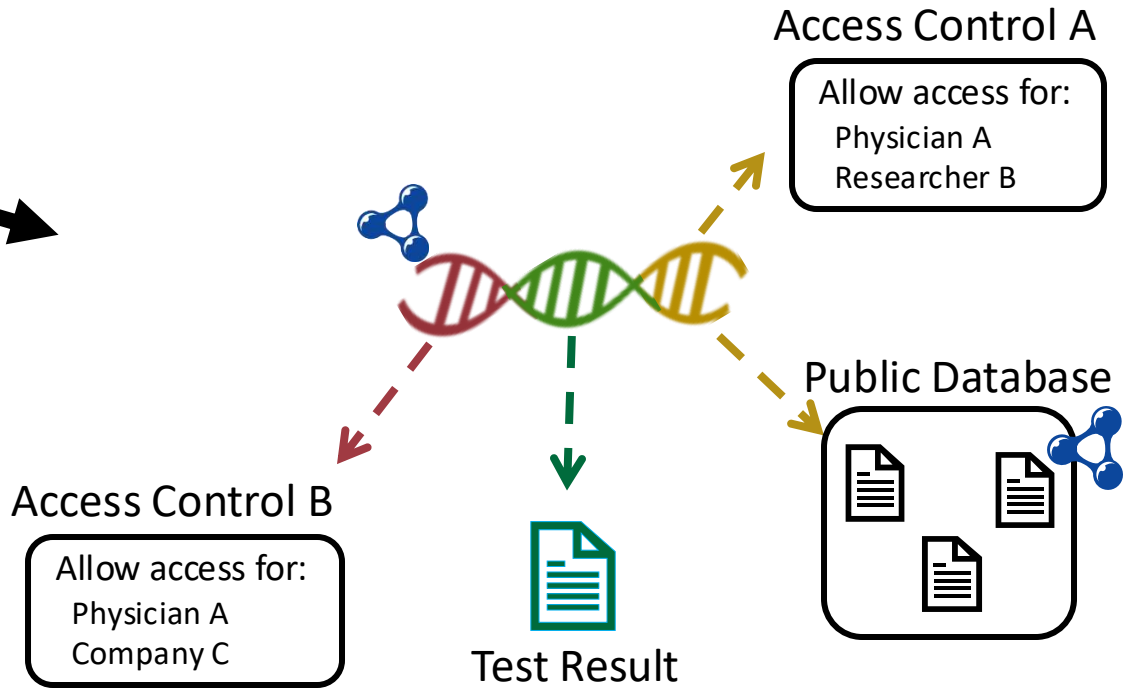
2. Genomic Data Representation

From a file-based approach

To a granular, dynamic RDF approach



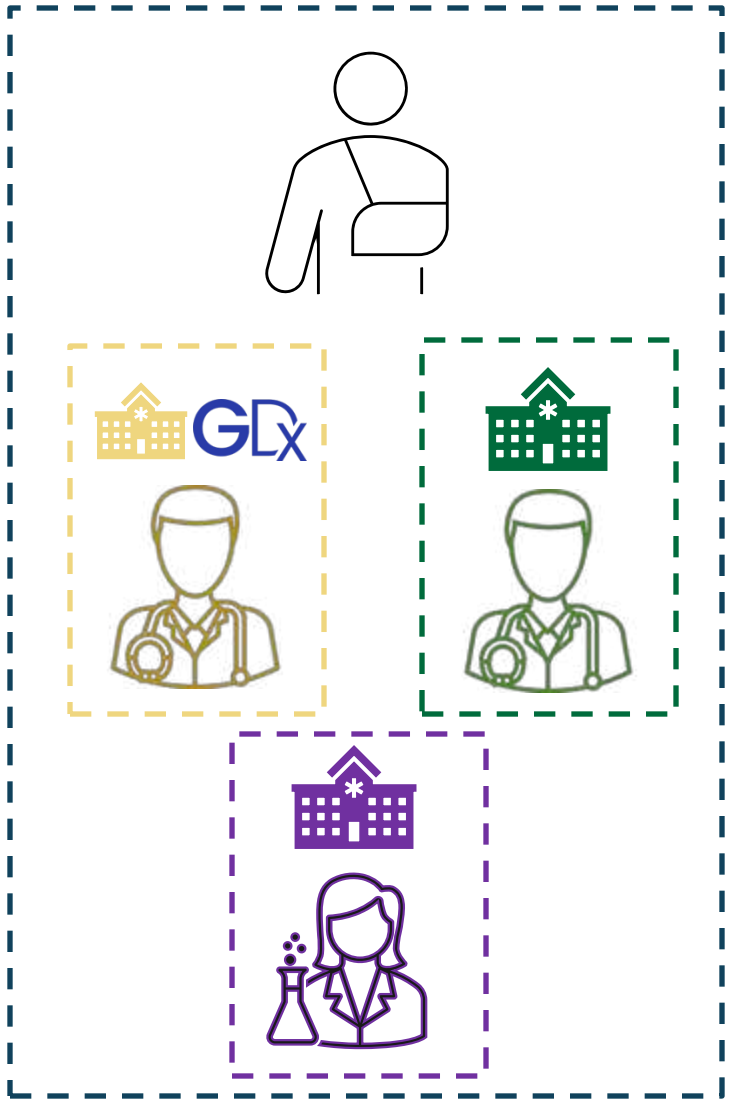
Conversion using existing ontologies

Policy definition via ODRL[1]

Implementation within Solid Pod Env

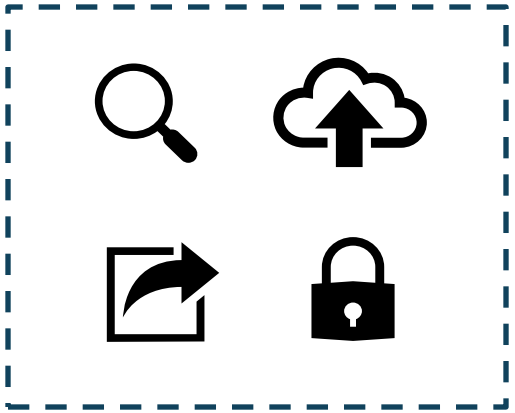
[1] https://www.w3.org/TR/odrl-model/

Future Possibilities?

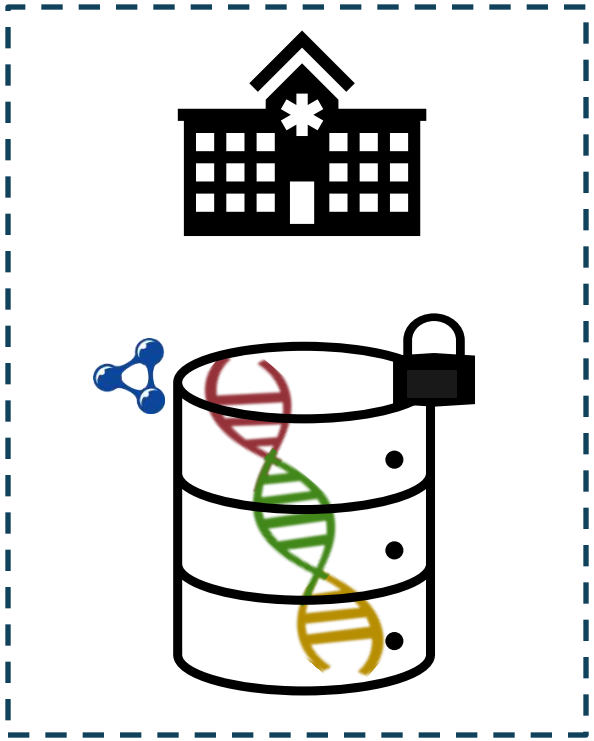


Data Users

Technological Infrastructure



Data Storage



MORE dynamic

MORE patient in-the-loop

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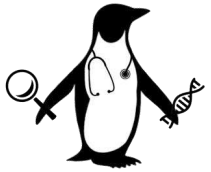
Thank you!



More PhD specifics

LinkedIn

GitHub

Advisors: Dr. Gökhan Ertaylan[1], Dr. Bart Buelens[1], Dr. Ruben Taelman[2], Prof. Dr. Ruben Verborgh[2] 1 = vito 2 = GHENT UNIVERSITY

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Informal Demo / Tutorial Later Today

(for those interested)

Solid Cockpit

knowledgeonwebscale.github.io/solid-cockpit/

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Supplemental Slides

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Genomic Data

Clinical-Grade

Higher read quality

More expensive

Stricter use-regulations

Research-Grade

Lower read quality

Less expensive

Flexible use-regulations

Dimension	Clinical Grade Genome Data Sharing	Research Grade Genome Data Sharing
1. Primary Purpose and Scope	Patient care and medical decision-making.	Discovery and hypothesis-driven studies.
2. Data Accuracy and Quality	Stringent quality control (QC) and validation.	High but no absolute QC standards.
3. Regulatory Requirements	Subject to medical regulatory frameworks.	Subject to research ethics regulations.
4. Consent Requirements	Clinical-informed consent.	Research-informed consent.
5. Privacy and Confidentiality	Highest level of patient privacy required.	Strong but variable privacy measures.
6. Security Controls	Robust, enterprise-grade security.	Secure but more flexible.
7. Data Governance and Oversight	Hospital/clinic-based governance.	Institution or consortium-based governance.
8. Ethical Considerations	Focus on direct patient benefit and “do no harm.”	Focus on scientific advancement and participant welfare.
9. Infrastructure Requirements	Highly reliable, clinically validated systems.	Flexible, scalable research infrastructure.
10. Architecture Requirements	Standardized, interoperable clinical systems.	Flexible, modular architectures.
11. Legal Liabilities	High.	Moderate.
12. Interoperability	High necessity for clinical integration.	Variable.
13. Data Ownership and Control	Patient-oriented ownership with institutional custodianship.	Institution or sponsor-owned.
14. Data Sharing Scope	Limited to necessary clinical stakeholders.	Broad or consortium-based.
15. Cost and Funding Models	Healthcare reimbursement or institutional budgets.	Grant-funded or institutional research budgets.

Elixir Forum 2026

Genomic Sequence Data Sharing for Clinical Practice: A Scoping Review

vito

GHENT UNIVERSITY

Aim: Identify existing implementations, guidelines, and challenges of clinical sharing of genomic data.

Methods: Reviewed 55 studies, 57 national genomics projects, and 20 major genomics companies.

Results: Clinical genome sharing implementations exist, yet scalability and widespread adoption remain limited.

Suggested steps forward:

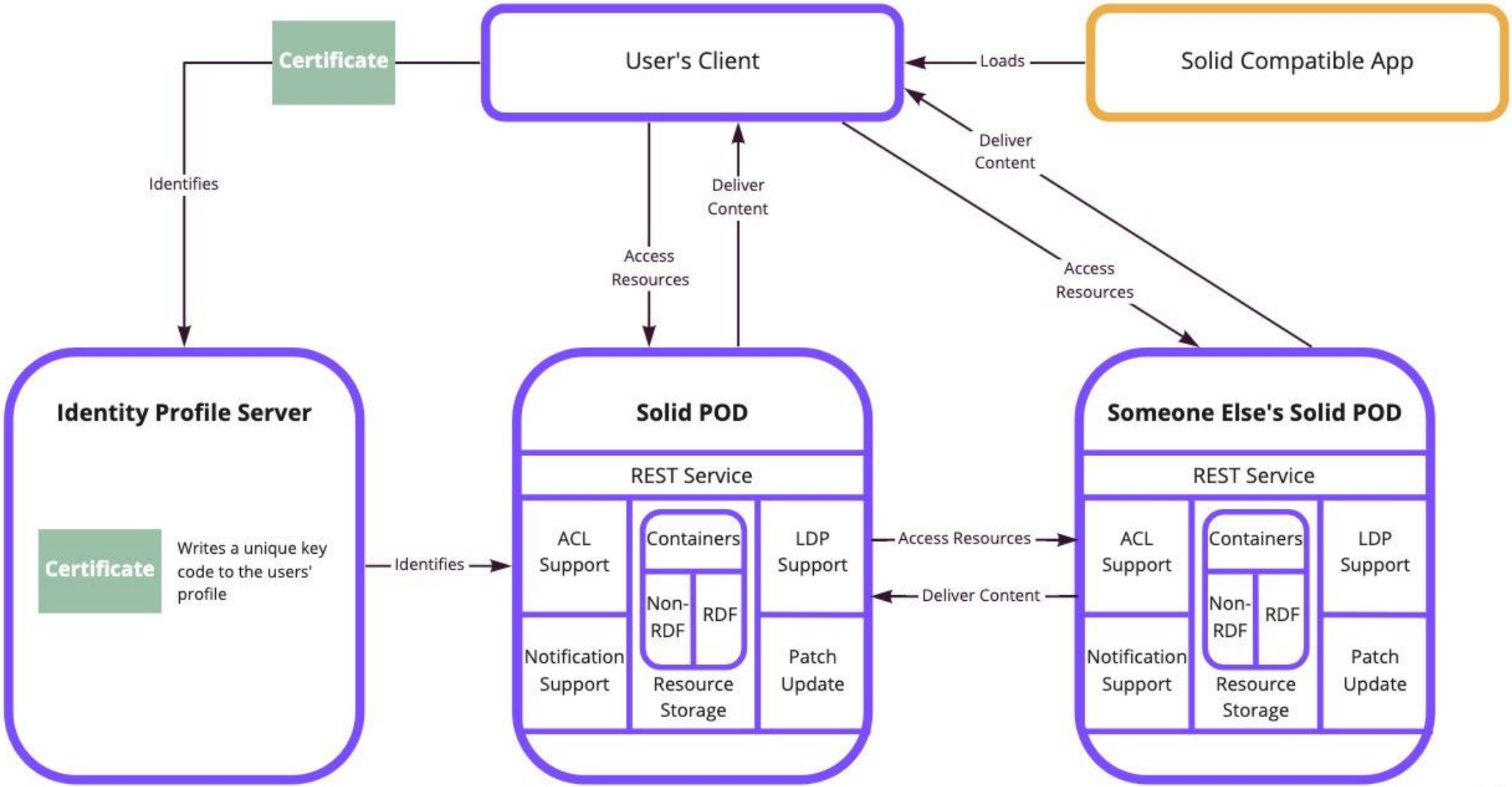
Harmonize legal bases for secondary clinical use of research data

Mandate (GA4GH) technical standards

Scale laboratory accreditation & continual pipeline validation

Align economic incentives

Solid's Architecture



https://solid.georgetown.domains/technical-component/

Solid Pod Implementations?

BBC MyPDS App

Flemish Data Utility Company

health data vaults

Flemish Smart Dataspace

Solid Lab Use-cases

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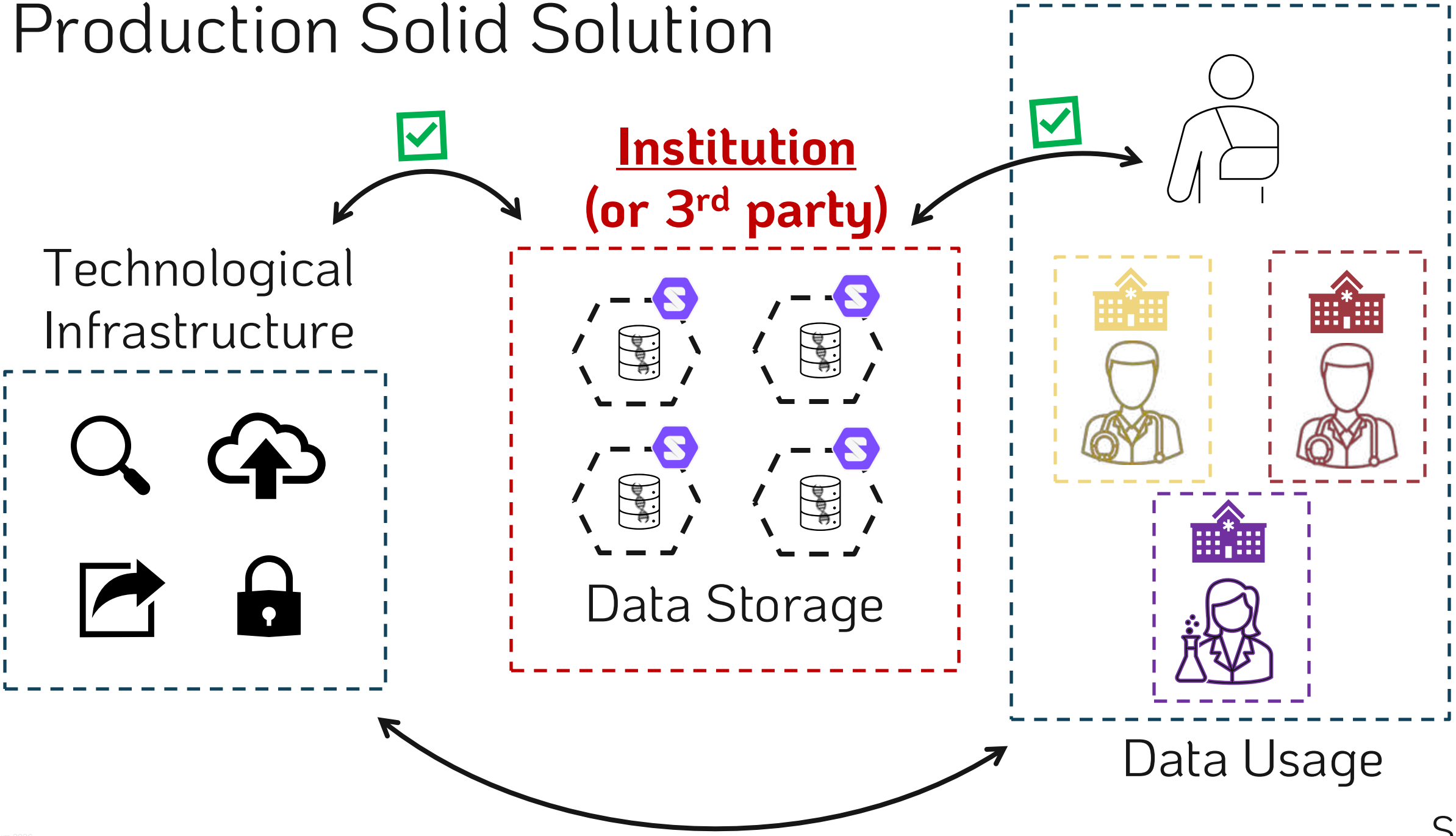
A Production Solid Solution



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Semantic VCF – *Sequencing Data Formats Intro*

FASTQ ✚ **FASTA** → **SAM/BAM** → **VCF**

Raw Sequencing Outputs

Contiguous DNA sequence

FASTQ reads aligned to a FASTA reference

Areas in SAM where sample and reference differ

```
1   >gi|568336023|gb|CM000663.2| Homo sapiens chromosome 1, GRCh
2   CCTAACCCTAACCCTAACCCTAACCCTAACCCTAACCCTAACCCTAACCCTAACCCTAAC
3   ACCCTAACCCTAACCCTAACCCTAACCCAACCCTAACCCTAACCCTAACCCTAACCCTAA
4   TAACCCTAACCCTAACCCTAACCCTAACCTAACCCTAACCCTAACCCTAACCCTAACCCT
5   CTAACCCTAACCCTAACCCTAACCCTAACCCTAAACCCTAAACCCTA
6   CAACCCCAACCCCAACCCCAACCCCAACCCCAACCCTAACCC
7   CCTAACCCTAACCCTAACCCTAACCCTAACCCCTAACCCCTA
8   TAACCCTAACCCCTAACCCTAACCCTAACCCTAACCCTCGCG
9   TCTGACCTGAGGAGAACTGTGCTCCGCCTTCAGAGTACCACC
10  CGCCCTCGCGGTGCTCTCCGGGTCTGTGCTGAGGAGAACGCA
11  CGGCGCAGGCGCAGAGAGGCGCGCCGCGCCGGCGCAGGCGCA
12  AGAGAGGCGCGCCGCGCCGGCGCAGGCGCAGAGAGGCGCGCC
13  CGCGCCGGCGCAGGCGCAGACACATGCTAGCGCGTCGGGGTG
14  GCCGCGCCGGCGCAGGCGCAGAGACACATGCTACCGCGTCCA
15  AGGCGCACCGCGCCGGCGCAGGCGCAGAGACACATGCTAGCG
16  GCAGAGACGCAAGCCTACGGGCGGGGGTTGGGGGGGCGTGTG
17  GCTGGGGCGGGGGGAGGGTGGCGCCGTGCACGCGCAGAAACT
18  GGTAGAACCTCAGTAATCCGAAAAGCCGGGATCGACCGCCCC
19  GCTTGCTCACGGTGCTGTGCCAGGGCGCCCCCTGCTGGCGAC
20  GAGTGGTGGCCAGCGCCCCCTGCTGGCGCCGGGGCACTGCAG
```

```
@FORJUSP02
CCGTCAATTC
+
AAAAAAAAAA
```

Q scores (as ASCII ch

Base=T, Q=':'=25

```
@HD     VN:1.0  SO
@SQ     SN:chr20
@PG     ID:TopHat
lign-edit-dist 2 -
20 /data/user446/m
HWI-ST1145:74:C101
       CCGTGTTTAAA
C    BBDCCDDCCDDDD
    AS:i:-15
HWI-ST1145:74:C101
       TGCTGGATCAT
G    DCDDDDEDDDDDD
    AS:i:-16
HWI-ST1145:74:C101DACXX:7:1204:14760:4030       16      chr20   270877  50      100M    *       0       0
       GGCTTTATTGGTAAAAAAGGAATAGCAGATTTAATCAGAAATTCCCACCTGGCCCAGCAGCACCAACCAGAAAGAAGGGAAGAAGACAGGAAAAAACCA
C    DDDDDDDDDCCDDDDDDDDDDEEEEEEEFFFEFFEGHHHHFGDJJIHJJIJIJJJIIIIGGFJJIHIIIIJJJJJJIGHHFAHGFHJHFGGHFFFDD@BB
    AS:i:-11        XM:i:2  XO:i:0  XG:i:0  MD:Z:0A85G13    NM:i:2  NH:i:1
HWI-ST1145:74:C101DACXX:7:1210:11167:8699       0       chr20   271218  50      50M4700N50M     *       0
       0       GTGGCTCTTCCACAGGAATGTTGAGGATGACATCCATGTCTGGGGTGCACTTGGGTCTCCGAAGCAGAACATCCTCAAATATGACCTCTCG
accepted_hits.sam
```

```
##fileformat=VCFv4.3
##fileDate=20090805
##source=myImputationProgramV3.1
##reference=file:///seq/references/1000GenomesPilot-NCBI36.fasta
##contig=<ID=20,length=62435964,assembly=B36,md5=f126cdf8a6e0c7f379d618ff66beb2da,species="Homo sapiens",taxonomy=x>
##phasing=partial
##INFO=<ID=NS,Number=1,Type=Integer,Description="Number of Samples With Data">
##INFO=<ID=DP,Number=1,Type=Integer,Description="Total Depth">
##INFO=<ID=AF,Number=A,Type=Float,Description="Allele Frequency">
##INFO=<ID=AA,Number=1,Type=String,Description="Ancestral Allele">
##INFO=<ID=DB,Number=0,Type=Flag,Description="dbSNP membership, build 129">
##INFO=<ID=H2,Number=0,Type=Flag,Description="HapMap2 membership">
##FILTER=<ID=q10,Description="Quality below 10">
##FILTER=<ID=s50,Description="Less than 50% of samples have data">
##FORMAT=<ID=GT,Number=1,Type=String,Description="Genotype">
##FORMAT=<ID=GQ,Number=1,Type=Integer,Description="Genotype Quality">
##FORMAT=<ID=DP,Number=1,Type=Integer,Description="Read Depth">
##FORMAT=<ID=HQ,Number=2,Type=Integer,Description="Haplotype Quality">
#CHROM POS     ID        REF    ALT     QUAL FILTER INFO                              FORMAT      NA00001        NA00002        NA00003
20     14370   rs6054257 G      A       29   PASS   NS=3;DP=14;AF=0.5;DB;H2           GT:GQ:DP:HQ 0|0:48:1:51,51 1|0:48:8:51,51 1/1:43:5:.,.
20     17330   .         T      A       3    q10    NS=3;DP=11;AF=0.017               GT:GQ:DP:HQ 0|0:49:3:58,50 0|1:3:5:65,3   0/0:41:3
20     1110696 rs6040355 A      G,T     67   PASS   NS=2;DP=10;AF=0.333,0.667;AA=T;DB GT:GQ:DP:HQ 1|2:21:6:23,27 2|1:2:0:18,2   2/2:35:4
20     1230237 .         T      .       47   PASS   NS=3;DP=13;AA=T                   GT:GQ:DP:HQ 0|0:54:7:56,60 0|0:48:4:51,51 0/0:61:2
20     1234567 microsat1 GTC    G,GTCT  50   PASS   NS=3;DP=9;AA=G                    GT:GQ:DP    0/1:35:4       0/2:17:2       1/1:40:3
```

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What does a VCF File Look like?

Example

VCF header

```
##fileformat=VCFv4.0
##fileDate=20100707
##source=VCFtools
##reference=NCBI36
##INFO=<ID=AA,Number=1,Type=String,Description="Ancestral Allele">
##INFO=<ID=H2,Number=0,Type=Flag,Description="HapMap2 membership">
##FORMAT=<ID=GT,Number=1,Type=String,Description="Genotype">
##FORMAT=<ID=GQ,Number=1,Type=Integer,Description="Genotype Quality (phred score)">
##FORMAT=<ID=GL,Number=3,Type=Float,Description="Likelihoods for RR,RA,AA genotypes (R=ref,A=alt)">
##FORMAT=<ID=DP,Number=1,Type=Integer,Description="Read Depth">
##ALT=<ID=DEL,Description="Deletion">
##INFO=<ID=SVTYPE,Number=1,Type=String,Description="Type of structural variant">
##INFO=<ID=END,Number=1,Type=Integer,Description="End position of the variant">
```

Body

#CHROM	POS	ID	REF	ALT	QUAL	FILTER	INFO	FORMAT	SAMPLE1	SAMPLE2
1	1	.	ACG	A,AT	.	PASS	.	GT:DP	1/2:13	0/0:29
1	2	rs1	C	T,CT	.	PASS	H2;AA=T	GT:GQ	0|1:100	2/2:70
1	5	.	A	G	.	PASS	.	GT:GQ	1|0:77	1/1:95
1	100		T	<DEL>	.	PASS	SVTYPE=DEL;END=300	GT:GQ:DP	1/1:12:3	0/0:20

Mandatory header lines

Optional header lines (meta-data about the annotations in the VCF body)

Reference alleles (GT=0)

Alternate alleles (GT>0 is an index to the ALT column)

Deletion

SNP

Large SV

Insertion

Other event

Phased data (G and C above are on the same chromosome)

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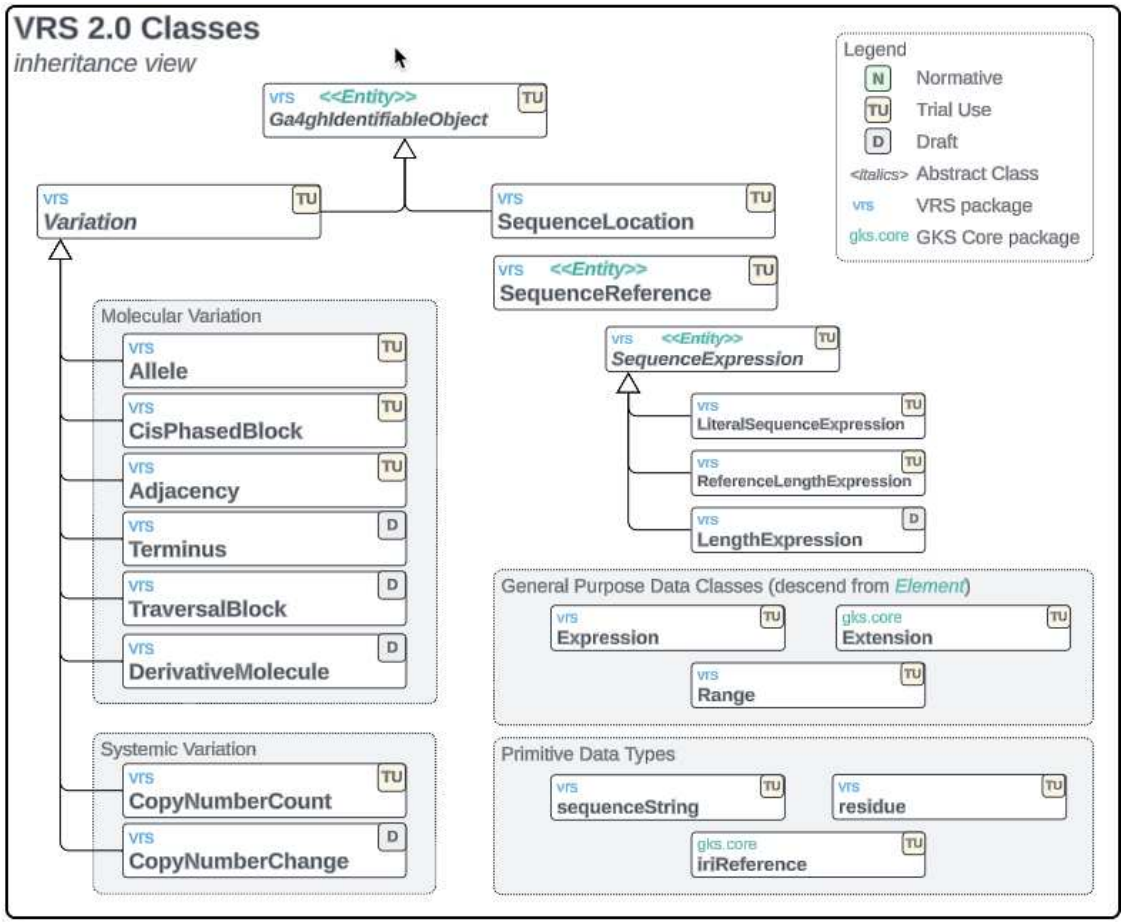
https://davetang.github.io/learning_vcf_file/

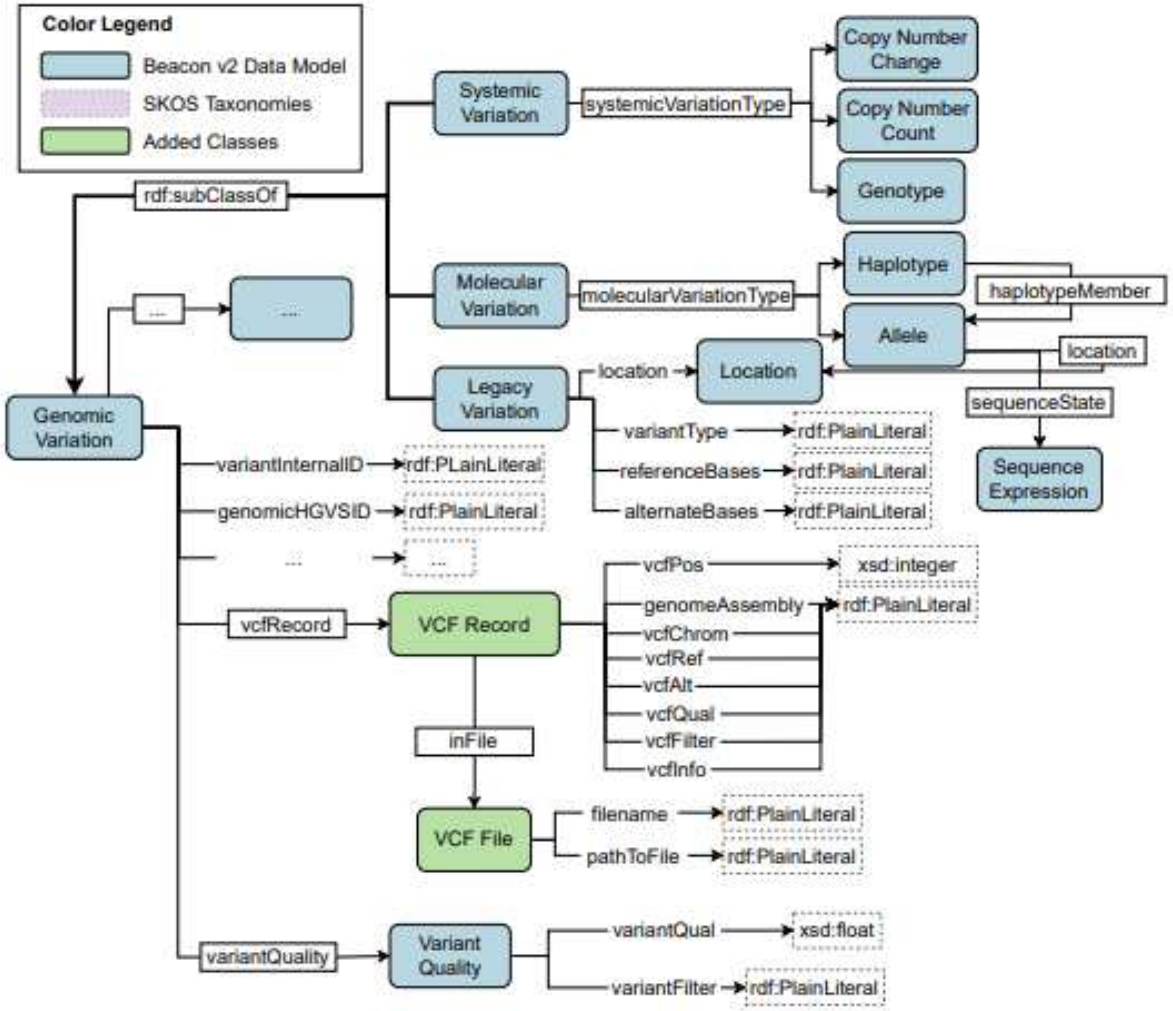
VCF Spec: https://samtools.github.io/hts-specs/VCFv4.2.pdf

Representing VCF Semantically?

GA4GH Variant Schema:



HERO Genomics Ontology:



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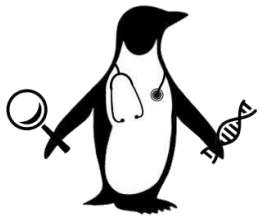
https://vrs.ga4gh.org/en/2.0.0-ballot.2024-11/appendices/class_diagram.html#classdiagram

https://www.dei.unipd.it/~silvello/papers/2025_MCS_SWAT4HCLS.pdf

Envisioned Product Evolution



PhD End | Short Term | Medium Term | Long Term

PhD Deliverable

Start-up / Product development
+
Hospital implementation

Integrate with EHR
+
Increase application domains

Increase 3rd party application support
+
Scale to Europe and beyond

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